



Laboratory Investigation Report

Patient Name	: Jaweera	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 24 Y 0 M 0 D /F	OP/IP No	: OP/127671
Max ID/Mobile	: 542581/8752005200	Collection Date/Time	: 11/Feb/2022 04:16PM
Lab ID	: 1266022200214	Receiving Date	: 11/Feb/2022
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 12/Feb/2022
Passport No.	:		

Immunoassay

Total - Thyroid Profile*, Serum

Date	11/Feb/2022 04:16PM	Unit	Bio Ref Interval
T3 (Total) CLIA	1.97	ng/mL	0.87-1.78
T4 (Total) CLIA	16.29	µg/dL	6.09-12.23
TSH Chemiluminescence	7.84	uIU/ml	0.34-5.6

Comment

Parameter	Unit	Cord Blood	Adult	1st Trimester	2nd Trimester	3rd Trimester
TSH	uIU/ml	2.3 - 13.2	0.38 - 5.33	0.1 - 2.5	0.2 - 3.0	0.3 - 3.0

Increased in primary Hypothyroidism.
Decreased in primary Hyperthyroidism

Total Thyroid Profile : (Thyroid Function Test, TFT)
T3 (Total), Triiodothyronine
Increase in Hyperthyroidism, and T3 toxicosis,
Decreased in hypothyroidism, states with decreased TBG, and acute and subacute non thyroidal illness
T4(Total) Thyroxine
Increased in Hyperthyroidism, states with increased TBG, Thyrotoxicosis
Decreased in Hyperthyroidism, states with decreased TBG and Strenuous exercise
TSH, Serum : Thyrotropin(Thyroid Stimulating Hormone)
Increased in primary Hypothyroidism.
Decreased in primary Hyperthyroidism.

Note : TSH levels are subject to circadian variation, reaching peak levels between 2 – 4 am and at a minimum between 6 – 10 pm. The variation is of the order of 50% - 206 %, hence time of the day has influence on the measured serum TSH concentrations.
TSH assay is strandized to the 3rd generation for human TSH.
The Cyclical variations may be quite large; therefore the timing of specimen collection must be strictly controlled.
Advise : Kindly do Thyroid Profile/TSH in morning hours only.



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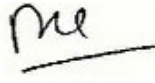
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Immunoassay

Kindly correlate with clinical findings

***** End Of Report *****

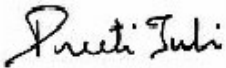
Dr. Poonam. S. Das, M.D.
Principal Director-
Max Lab & Blood Bank Services



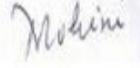
Dr. Dilip Kumar M.D.
Associate Director &
Manager Quality



Dr. Nitin Dayal, M.D.
Principal Consultant & Head,
Haematopathology



Dr. Preeti Tuli, M.D.
Sr. Consultant - Lab Medicine



Dr. Mohini Bhargava, MD
Principal consultant (Biochemistry)



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Hematology Special

Test Name	Result	Unit	Bio Ref Interval
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PDF Attached

Haemoglobin HPLC,EDTA

Lab Number	1479/2022		
Hb F	0.7	%	0.0-1.0
P2	3.7		
P3	5.4	%	<6.0
HbA0 (non-glycated)	86.9	%	80-90
Hb A2	2.4	%	2.0-3.5

RBC Indices

Haemoglobin.	10.0	gm/dl	12-15
Non- Haemoglobincyanide			
RBC Count.	3.97	10~12/L	4.5-5.5
Electrical Impedance			
MCV.	80.6	fL	83-101
Electrical Impedance			
MCH.	25.1	pg	27-32
Calculated			
RDW.	22.7	%	11.5-14.5
Calculated			

Interpretation (HPLC)

Normal Study

Comment

(Syn: - High Performance Liquid Chromatography, Hb HPLC)

Hb HPLC is used for screening of Beta Thalassemia and detection of many other haemoglobin variants (Hemoglobin E, D Punjab, Sickle cell anaemia etc.) along with their concentration to establish zygosity.

Normal HbA2 values can be seen in Alpha Thalassemia trait, silent/ atypical Beta Thalassemia and Beta Thalassemia with iron deficiency.

A repeat Hb HPLC should be performed after replenishing iron stores in cases with microcytic hypochromic indices.

Advise: -

If there is absence of iron deficiency & persistence of microcytic hypochromic indices, then the possibility of Alpha Thalassemia trait or silent/ atypical Beta Thalassemia trait cannot be ruled out. Hb HPLC of parents and molecular sequencing of globin genes are recommended in these cases.



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Hematology Special

Test Name	Result	Unit	Bio Ref Interval
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In individuals with beta thalassemia trait and carriers of haemoglobin variants –

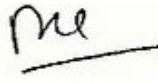
1. Familial study is recommended.
2. In cases of young/ pregnant females - Hb HPLC testing of spouse is strongly recommended.
3. Genetic counselling

Kindly correlate with clinical findings

*** End Of Report ***



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Clinical Biochemistry

KFT Profile with Calcium,Uric Acid, Serum

Date	11/Feb/2022 04:16PM	Unit	Bio Ref Interval
Urea Enzymatic Rate (Urease)	25.9	mg/dL	17.12 - 55.64
Creatinine Alkaline picrate kinetic	0.54	mg/dL	0.6 - 1.1
eGFR MDRD	138.70	ml/min/1.73 m ²	
Uric Acid Uricase, Colorimetric	4.72	mg/dl	2.6 - 6.0
Calcium (Total) Arsenazo III	9.56	mg/dl	8.9 - 10.3
Sodium ISE Indirect	137.6	mmol/L	136 - 144
Potassium ISE Indirect	4.19	mmol/L	3.6 - 5.1
Chloride ISE Indirect	103.4	mmol/l	101-111
Bicarbonate Colorimetric, PEP-C	23.5	mmol/l	22-29

Interpretation Ref. Range

eGFR - Estimated Glomerular Filtration Rate is calculated by MDRD equation which is most accurate for GFRs $\leq$ 60ml / min /1.73 m².MDRD equation is used for adult population only.

<60ml / min / 1.73 m² - Chronic Kidney Disease

<15 ml / min /1.73 m² - Kidney failure



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Clinical Biochemistry

Liver Function Test Profile,Serum

Date	11/Feb/2022 04:16PM	Unit	Bio Ref Interval
Total Protein Biuret	8.56	g/dl	6.5 - 8.1
Albumin BCP	4.5	g/dl	3.5 - 5.0
Globulin Calculated	4.1	g/dl	2.3 - 3.5
A.G. ratio Calculated	1.1		1.2 - 1.5
Bilirubin (Total) Diazo	1.38	mg/dl	0.3 - 1.2
Bilirubin (Direct) Diazo	0.24	mg/dl	0.1 - 0.5
Bilirubin (Indirect) Calculated	1.14	mg/dL	0.1 - 1.0
SGOT- Aspartate Transaminase (AST) UV without P5P	24	U/L	< 50
SGPT- Alanine Transaminase (ALT) Kinetic Rate using LDH	14	U/L	17 - 63
Alkaline Phosphatase PNP AMP Buffer	74	U/L	32 - 91
GGTP (Gamma GT), Serum Enzymatic Rate	10.0	U/L	7 - 50



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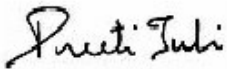
Clinical Biochemistry**Blood Sugar (Random), Fluoride Plasma**

Date	11/Feb/2022 04:16PM	Unit	Bio Ref Interval
Random Glucose UV-Hexokinase	80.9	mg/dl	74 - 140

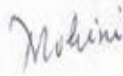
Interpretation A random plasma glucose of > 200 mg/dl with symptoms of diabetes is diagnostic. A confirmatory fasting plasma Glucose (FPG) test or OGTT should be completed on a different day if the clinical condition of the patient permits.

Kindly correlate with clinical findings

*** End Of Report ***



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Principal consultant (Biochemistry)



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Hematology

APTT (Partial Thromboplastin Time), Citrate Plasma, Sodium Citrate

Date	11/Feb/2022 04:16PM	Unit	Bio Ref Interval
APTT	40.5	Sec	25.3 - 34.3
Control	29.8	Sec	

Interpretation

(Activated Partial Thromboplastin Time; PTT)

APTT is the test which checks the “intrinsic coagulation” pathway and is useful for detecting screening of haemophilia A & B, screening of coagulation inhibitors like lupus anticoagulant.

APTT can also be used to monitor heparin monitoring.

Raised APTT is found in - Liver disease, DIC, heparin treatment, circulating inhibitors (Lupus Anticoagulant), and haemophilia (deficiency of Factor VIII & IX).

Low APTT may be seen in - high concentration Factor VIII and is independent risk factor for increased incidence of thrombus formation.

Advice: - ‘APTT mixing study’, ‘Lupus Anticoagulant test’, ‘specific Factor(s) assay’ may be added on for further evaluation.

Prothrombin Time (with INR), Citrate Plasma

Date	11/Feb/2022 04:16PM	Unit	Bio Ref Interval
Prothrombin Time (PT) Photo-Optical-Nephelometry	11.0	Sec	9.7 - 13.3
MNPT Value	11.9	Sec	
INR	0.92		

Interpretation

(Syn: - Prothrombin Time)

PT is the test which checks the “extrinsic coagulation” pathway and is useful for detecting coagulation deficiency, liver disease and disseminated intravascular Coagulation (DIC).

PT can also be expressed as International normalized ratio (INR) used for monitoring warfarin therapy.

Raised PT value seen in - factor deficiency (Fibrinogen (I), Prothrombin (II), factor V, VII, X), oral anticoagulation therapy, liver diseases, Vitamin K deficiency and DIC.

Advice: - ‘PT mixing study’, ‘specific factor(s) assay’ may be added on for further evaluation.

CBC (Complete Blood Count), Whole Blood EDTA

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Hematology

04:16PM

Haemoglobin	9.9	g/dl	12.0 - 15.0
Modified cyanmethemoglobin			
Packed Cell, Volume	31.6	%	40-50
Calculated			
Total Leucocyte Count (TLC)	4.9	10~9/L	4.0-10.0
Electrical Impedance			
RBC Count	4.07	10~12/L	3.8-4.8
Electrical Impedance			
MCV	77.7	fL	83-101
Electrical Impedance			
MCH	24.2	pg	27-32
Calculated			
MCHC	31.2	g/dl	31.5-34.5
Calculated			
Platelet Count	159	10~9/L	150-410
Electrical Impedance			
MPV	12.2	fl	7.8-11.2
Calculated			
RDW	21.5	%	11.5-14.5
Calculated			

Differential Cell Count

VCS / Light Microscopy

Neutrophils	66.4	%	40-80
Lymphocytes	23.6	%	20-40
Monocytes	7.0	%	2-10
Eosinophils	2.4	%	1-6
Basophils	0.6	%	0-2

Absolute Leukocyte Count

Calculated from TLC & DLC

Absolute Neutrophil Count	3.25	10~9/L	2.0-7.0
Absolute Lymphocyte Count	1.2	10~9/L	1.0-3.0
Absolute Monocyte Count	0.34	10~9/L	0.2-1.0
Absolute Eosinophil Count	0.12	10~9/L	0.02-0.5
Absolute Basophil Count	0.03	10~9/L	0.02-0.1



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SIN No: B2B1504197, Test Performed at : 969 - Max Hospital, Patparganj, 108A, IP Ext, I.P.Extension, Patparganj, Delhi, 11

Booking Centre : 1534 - Arctern Healthcare Noida, 2nd Floor, Powerstation by Oyo Pioneer Square, Sector 62, Gurugram, Haryana 122002, 8752005200

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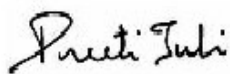
**Laboratory Investigation Report**

Patient Name	: Jaweera	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 24 Y 0 M 0 D /F	OP/IP No	: OP/127671
Max ID/Mobile	: 542581/8752005200	Collection Date/Time	: 11/Feb/2022 04:16PM
Lab ID	: 1266022200214	Receiving Date	: 11/Feb/2022
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 12/Feb/2022
Passport No.	:		

Hematology

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Preeti Tuli, M.D.
Principal Consultant & Quality Manager
Pathology.



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Immunoassay

Prolactin, Serum

Date	11/Feb/2022 04:16PM	Unit	Bio Ref Interval
Prolactin CLIA	8.64	ng/mL	

Ref Range

Males :	2.64 - 13.13
Females :	
Premenopausal (<50 years of age):	3.34 - 26.74
Postmenopausal (>50 years of age):	2.74 - 19.64

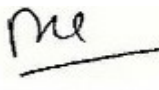
Interpretation

Increased in prolactin-secreting pituitary tumors, amenorrhea and/or galactorrhea, Chiari-Frommel and Argonz Del Cstillo syndromes, various types of hypothalamic-pituitary disease (e.g. sarcoidosis, granulomatous diseases, craniopharyngioma, metastatic cancer, empty sella syndrome), primary hypothyroidism, anorexia nervosa, polycystic ovary syndrome, renal failure, insulin-induced hypoglycemia, chest wall injury, adrenal insufficiency, and pituitary stalk section surgery
Decreased in pituitary apoplexy (Sheehan's Syndrome)

Kindly correlate with clinical findings

*** End Of Report ***


Dr. Poonam S. Das, M.D.
Principal Director-
Max Lab & Blood Bank Services


Dr. Dilip Kumar M.D.
Associate Director &
Manager Quality


Dr. Nitin Dayal, M.D.
Principal Consultant & Head,
Haematopathology



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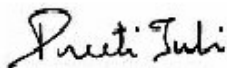
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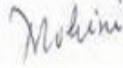
Laboratory Investigation Report

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Immunoassay



Dr. Preeti Tuli, M.D.
Sr. Consultant - Lab Medicine



Dr. Mohini Bhargava, MD
Principal consultant (Biochemistry)



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Serology			
Test Name	Result	Unit	Bio Ref Interval



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Laboratory Investigation Report

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Serology

Test Name	Result	Unit	Bio Ref Interval
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HIV Test (I and II),Serum

CLIA

HIV (I and II) Negative

Interpretation

- 1) A Negative result implies that no Anti HIV-1 or HIV-2 antibodies have been detected in the sample by this method. This means that either the patient has not been exposed to HIV-1 or HIV-2 infection or the sample has been taken during the "WINDOW PERIOD" (before the development of detectable levels of antibodies).
- 2) Positive result suggests the possibility of HIV-1 or HIV-2 infection.

Advise:

To rule out false positivity, false negativity and window period, kindly perform "Confirmatory Tests" like HIV I RNA (Qualitative) Real Time PCR.



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Test Name	Serology Result	Unit	Bio Ref Interval
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Hepatitis B Surface Antigen, Serum

CLIA

HBsAg	Negative		
HBsAg Test Value	0.39		0 - 0.90

Ref. Range

Negative	< 0.90
Borderline	0.90 - 5.0
Positive	> 5.0

Interpretation

- This test is used to detect hepatitis B surface antigen (HBsAg) in serum sample based on VITROS immunometric immunoassay technique and aid in the laboratory diagnosis of HBV infection.
- Viral hepatitis is a major public health problem with an estimated 257 million persistent carriers of hepatitis B virus (HBV) worldwide. Infection with HBV results in a wide spectrum of acute and chronic liver diseases that may lead to cirrhosis and hepatocellular carcinoma.
- Transmission of HBV occurs by percutaneous exposure to blood products, needle stick injury, sexual contact and perinatally from HBV-infected mothers to baby.
- Hepatitis B surface antigen (HBsAg), derived from the viral envelope, is the first antigen to appear following infection.
- Positive results should be correlated with other potential laboratory abnormalities and clinical picture.
- A negative test result does not exclude the possibility of exposure to or infection with hepatitis B virus.
- Levels of HBsAg may be undetectable both in early infection and late after infection.
- In rare cases HBsAg tests do not detect certain HBV mutant strains.
- HBs Ag disappears with recovery from clinical disease in most patients, however, it persists for years in carriers.



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Test Name	Serology Result	Unit	Bio Ref Interval
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VDRL/RPR, Serum

RPR(Syphilis) Non Reactive
Slide Flocculation

Comment

Interpretation

1. It is a screening test for syphilis which is useful for following the progression of disease and response to therapy. Rising titers are of immense value in confirming the diagnosis.
2. Biological false positive reactions exhibit low titers and are seen in conditions like Viral fevers, Mycoplasma infection, Chlamydia infection, Malaria, Immunizations, Pregnancy, Autoimmune disorders & past history of Treponemal infection.
3. It is advisable to confirm the diagnosis by tests such as TPHA & FTA-ABS.
4. A definitive clinical diagnosis should not be made by result of a single test only, but should be made by taking clinical history and other laboratory findings in to account.

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Ranjana Chhabra, M.D.
Senior Consultant Microbiology



Dr. Suchitra Jain, M.D.
Senior Consultant Microbiology



Dr. Ranjana Chhabra, M.D.
Senior Consultant Microbiology



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Serology			
Test Name	Result	Unit	Bio Ref Interval



Dr. Saloni Sehgal (MBBS, MD)
Principal Consultant &
Head Microbiology & Infection Control



Dr. Neera Kaushik
Senior Microbiologist



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Patient Data

Sample ID: B2B1504197
Patient ID: 1777845/1479/22
Name:
Physician:
Sex:
DOB:
Comments:

Analysis Data

Analysis Performed: 12/02/2022 12:43:29
Injection Number: 66410
Run Number: 237
Rack ID:
Tube Number: 3
Report Generated: 12/02/2022 13:17:16
Operator ID:

Peak Name	Calibrated Area %	Area %	Retention Time (min)	Peak Area
F	0.7	---	1.10	8212
Unknown	---	1.1	1.26	13481
P2	---	3.7	1.35	46342
P3	---	5.4	1.73	68172
Ao	---	86.9	2.48	1097430
A2	2.4	---	3.63	29167

Total Area: 1,262,804

F Concentration = 0.7 %
A2 Concentration = 2.4 %

Analysis comments:

